

# Siya Health®

---

**In medical school, I watched a brilliant, clever friend struggle for years — distracted, restless, unable to finish what he started — before he was finally diagnosed with ADHD. He's now a successful interventional cardiologist. That gap, between clever-but-struggling and diagnosed-and-thriving, is what we built Siya to close.**

Physician-led telehealth · care that follows through, before and after the visit · multi-state operations

## The problem

**The doctor makes the right care plan.  
The patient doesn't always get to live it.**

Care breaks down across three phases — before the visit, during it, and after. Not because of missing technology. Because of missing follow-through.

**Pre-visit**

**At the visit**

**Post-visit**

## Pre-visit

**Before the visit, the wrong patients get through — and the right ones don't show up.**

- Poor eligibility review → mismatched expectations, refunds, chargebacks, bad reviews

- No personalized reminders → no-shows

### Fix

**Our concierge team reviews eligibility before booking and calls patients personally to confirm.**

### At the visit

**During the visit, the doctor is doing admin work instead of practicing medicine.**

- Incomplete intake → doctor manually gathers history

- No documentation/order support → less patient-facing time

### Fix

**Our concierge team pre-builds the intake summary and scribes live, so the doctor's time is spent only on the patient.**

## Post-visit

# After the visit, care quietly falls apart.

- Pharmacy coordination failures → prescriptions don't reach the patient
- No structured follow-up → low compliance, interrupted care

## Fix

**Our concierge team owns pharmacy coordination and structured follow-up — checking in, explaining what to expect, catching problems before they interrupt care.**

Today, we've solved this with trained people operating disciplined protocols on top of existing tools. As those protocols mature, they become the blueprint for technology we build ourselves — proven first, encoded second.

Traction

Revenue growing steadily since February.  
Cash-flow positive.

2,700+

Patients treated (growing)

1,200+

Neurocognitive evaluations

4 states

Multi-state operations

Cash-flow+

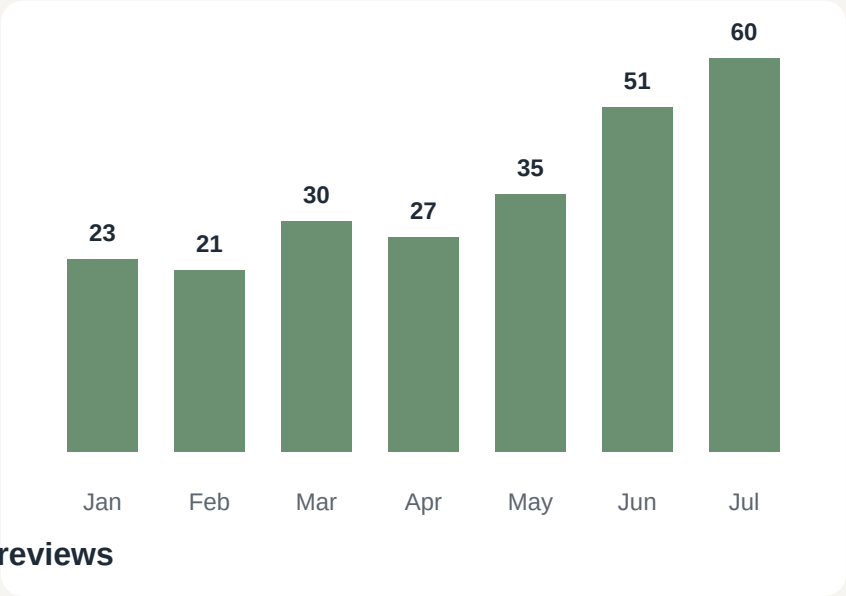
Operations

Reviews: 4.9★ on Google (100+ reviews) · 4.75★ average across 600+ third-party provider reviews

No-shows: Declining trend Q4 2025–Q1/Q2 2026 as trained MAs and automated reminders took effect — now plateaued, pointing to factors beyond reminders we're actively addressing next.

[Retention: 3-month cohort data pending final team calculation — leave placeholder, do not fabricate]

Monthly gross revenue — 2026



Where we are, honestly

## We saw the risk before you did — and we're already fixing it.

### Proof

Founder-led relationships filled every provider's panel, not just the founder's own. It worked — 2,700+ patients prove it.

### Risk

One person's time doesn't scale. We knew that going in.

### Fix

September 2026 — a fractional COO joined, already driving fundraising readiness and employer/B2B pipeline. This round adds the clinical capacity to match.

Business model

Cash-pay, recurring, no insurance risk.

|                                                                              |                                                                                               |                                                                                            |
|------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------|
| <div>\$149</div> <div>Initial evaluation</div> <div>One-time, self-pay</div> | <div>\$79 / mo</div> <div>Ongoing care — non-controlled</div> <div>Recurring membership</div> | <div>\$149 / mo</div> <div>Ongoing care — controlled</div> <div>Recurring membership</div> |
|------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------|

Siya Health Inc (MSO) + affiliated professional corporation(s) — standard, legally reviewed structure. B2B/employer pricing and MSO management-fee terms are being finalized as the company adds affiliated providers and employer partnerships.

150+ recurring members and growing. Cash-pay means no payer contracts, no reimbursement risk, revenue from day one.

Market — bottom-up

21.7M working adults live with a diagnosable condition affecting attention, sleep, or focus — most of them undiagnosed or untreated.

| Layer | Definition                                                                     | Size          |
|-------|--------------------------------------------------------------------------------|---------------|
| TAM   | US clinical services for adult ADHD + clinical insomnia                        | ~\$18B        |
| SAM   | Employed adults with either condition, telehealth-willing, cash-pay comparable | ~21.7M people |
| SOM   | Year-3, capacity-derived — 8–10 FTEs at ~800 patients each                     | ~\$3.8M–4.8M  |

Built bottom-up: 163M employed U.S. adults (BLS 2026) · 15.5M diagnosed adult ADHD, 63% employed · ~10% clinical insomnia disorder among employed adults · overlap-adjusted (44% ADHD/insomnia comorbidity, per published clinical data). Existing independent contractor network (8 providers) represents additional capacity not included in this Year-3 estimate. Full sizing methodology available in diligence.

Competition

Different job. Not a feature checklist.

|                                                         |                                                                                                                                 |
|---------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------|
| Employer mental-health platforms (e.g. Lyra Health)     | Broad coverage, not built for objective, condition-specific clinical depth                                                      |
| Generic access & counseling (EAPs, episodic telehealth) | Gets you seen once. No structured monitoring, no concierge follow-through                                                       |
| DTC brain-training / wellness apps                      | Games and self-assessment. Not clinical, no escalation if something real is found                                               |
| Siya                                                    | Clinical-grade screening + a trained concierge team + real escalation to licensed providers, on your schedule, not office hours |

No feature matrix. No 'everyone top-right' quadrant.

Go-to-market

Working channels today. Capacity is the job this round funds.

01 · Today

Channels that fill panels

Existing channels + founder-led fill — ~\$80 CAC estimate (current channel estimate, not yet a mature/audited figure).

02 · Already in motion

Fractional COO

Fractional COO since Sept 2026 — driving fundraising readiness and B2B pipeline, self-funded.

03 · This round's job

Clinical capacity

Add clinical capacity to serve proven demand.

04 · Timeline

Vendor-delivered CAC

Professional-vendor-driven CAC expected within ~6 months of engaging a partner — no calendar promise.

### Why now

**Demand is already here. Healthcare hasn't caught up to what working professionals actually need.**

While carefully screening ADHD patients, we noticed something worth building on: a negative ADHD or PTSD screen doesn't mean nothing is there. Healthcare screens for cognitive and mental health reactively — a PHQ flag in Epic, dementia screening starting at 65. We think tracking these markers longitudinally, especially for working professionals under sustained pressure, is worth building now.

**Discipline: SOPs, QA, and multi-state compliance already in place — the bottleneck is capacity and founder bandwidth, not the care model.**

## Use of funds

**We're raising \$500k to hold capacity and hand off acquisition.**

**01 Clinical capacity & retention · ~\$300k**

Two PA/NP hires (licensing, DEA, malpractice, benefits) + medical director multi-state supervision.

**02 Leadership capacity · ~\$100k**

Sustains the fractional COO retainer through the 18–24 month runway, already in motion since September 2026.

**03 Professional marketing partnership · ~\$100k**

Healthcare-specific vendor once capacity is stable.

**Objective: Stabilize capacity against proven demand, free founder bandwidth, then replace founder-led acquisition with a professional partner.**

## Founders

### Physicians who built the practice before asking for capital.



**Dr. Sneh Pandey**

Founder · CEO

- Board-certified physician
- 10,000+ patient encounters at Siya Health across primary care, obesity, hospital medicine, and ADHD care
- Built multi-state practice to 2,700+ patients, cash-flow positive, bootstrapped



**Dr. Swati Pandey**

Co-founder

- Board-certified physician, licensed in Pennsylvania
- Co-founder and clinical advisor — shapes clinical protocol standards across Siya Health

*I was strict about controlled substances in my own training, long before Siya existed. I'd read about companies getting sued for overprescribing, and I didn't want us to become another cautionary tale. So the pill counts, the random drug screens, the compliance formalities — none of that came from a regulation checklist. It came from not wanting my name attached to a company that cut corners on this.*

— Dr. Sneh Pandey

**Years spent learning how to deliver better care.  
This round scales what already works.**

Seed round

**\$500,000**

Clinical capacity & retention · Leadership capacity · Professional marketing partnership

**Siya Health®**

---

**Let's talk.**

**Dr. Sneh Pandey · Founder & CEO**

**chiefservant@siya.health**

siya.health